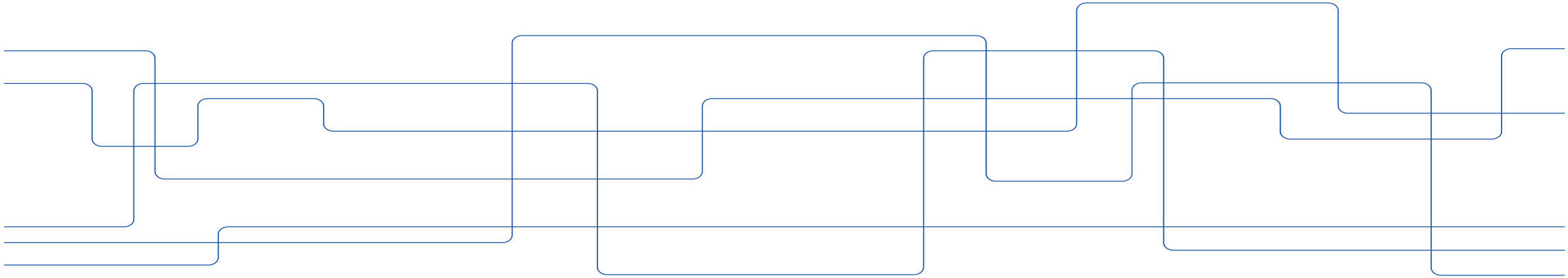
KTH ROYAL INSTITUTE OF TECHNOLOGY

Lecture 2: Paternalism and medical technologies

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Key points from previous lecture

Recap

- Applied ethics aims to provide normative guidance – e.g. *prescriptions* and *evaluations* regrading practical moral problems
- Normative claims need to be justified
  - To justify is to provide convincing reasons for a claim
- Reflective equilibrium – a coherentist theory
- Ethical theories (top-down, bottom-up, mid-level theory)
- Principlism – the four principles
- Applying Principlism



Make sure that you can: explain Principlism and the four principles and that you know how to apply that method.







What unites these public health strategies is that they are often brought up as examples of *paternalism*

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What is paternalism? Here is one definition:

"[T]he interference of a state or an individual with another person, against their will, and defended or motivated by a claim that the person interfered with will be better off or protected from harm."

(Dworkin 2014)

NB – it's the liberty or autonomy of the person that is interfered with.

Paternalistic interventions



SOFT

Interventions that help individuals achieve goals they have set for themselves.

(Haybron and Alexandrova 2013)

HARD

If individuals are prevented from or manipulated into, performing an action or making a choice on the basis that this will be in their own best interests.

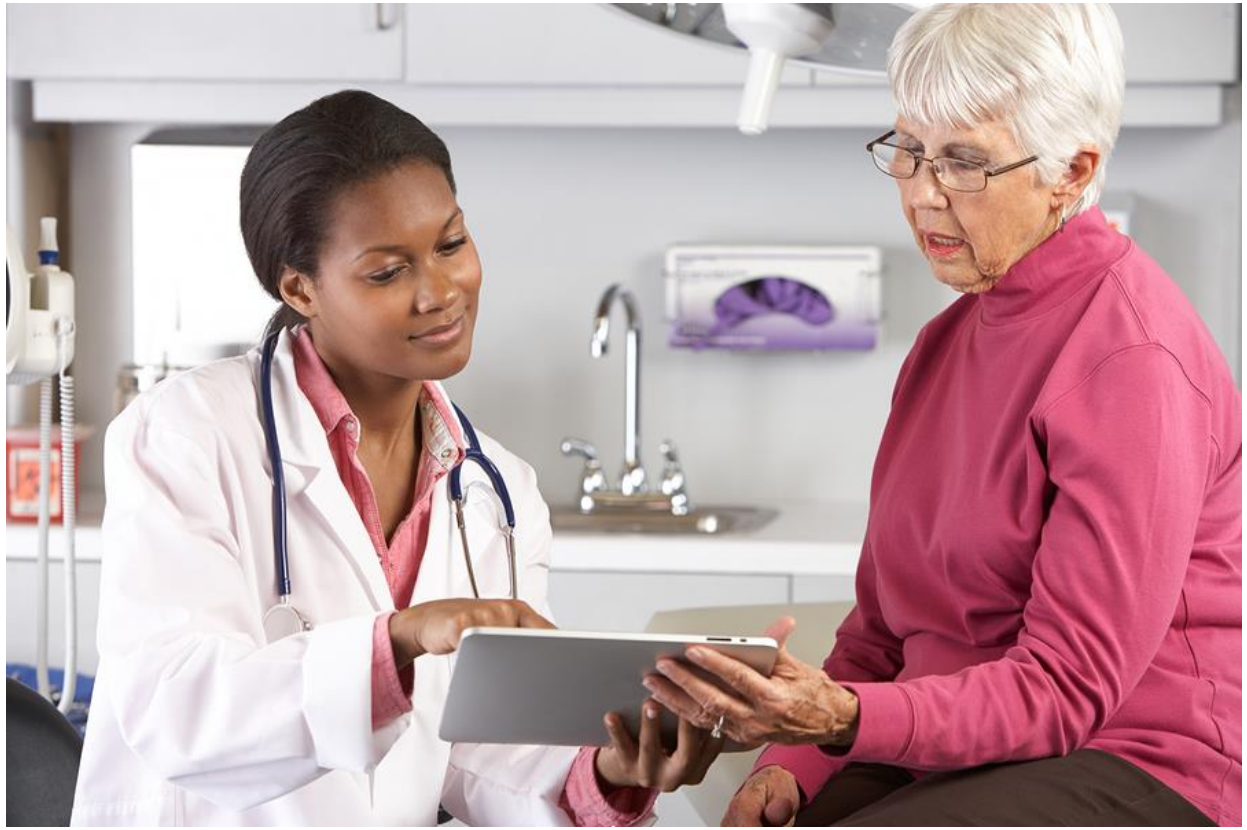
(Sometimes, the associated assumption is that they are not the best judge of their interests.)

(Begon 2016)

Medical paternalism

Paternalism has been one of the traditional characteristics of medicine and public health. It's been especially discussed in the context of the therapeutic relationship in medicine.

It implies that the physician makes decisions based on what they consider to be in the patient's best interests, even for those patients *who could make the decisions for themselves*.



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Criteria for medical paternalism

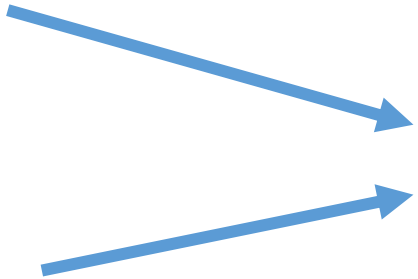
For an act to qualify as medical paternalism 1 & 2 below are jointly sufficient

1. a physician or other healthcare professional undertakes actions designed to limit the exercise of the patient's autonomy,
2. on the basis of clinical judgment and decision-making that are reliably beneficence based in order to protect the health-related interests of the patient from the consequences of the patient's decisions and actions based on them.

Note that *clinical judgement* in (2) is shorthand for: expert clinical judgements which are both evidence-based and beneficence-based in their evaluation of the relevant evidence.

Recall the four principles in Principlism

- **Autonomy**
- Nonmaleficence
- **Beneficence**
- Justice



Philosophical problem: when, if ever, are paternalistic interventions permissible?

Some examples of negative patient outcomes

Recall that: “Paternalism implies that the physician makes decisions based on what they consider to be in the patient’s best interests, even for those patients *who could make the decisions for themselves*.”

- So if we are to think of examples it might be tempting to point to some horrific practice like forced sterilization (some 60 000 cases in Sweden in the 1940s’ and 1950’s). Evidently those practices constituted a flagrant violation of human rights and ruined many peoples lives but strictly speaking that might not be medical paternalism, it does not meet the definition of clinical judgement nor of beneficence (*expected* positive outcome for that individual).
- A standard example of medical paternalism is withholding information from patients about their conditions (undermines autonomy and decision-making) or when the physician frames the options to influence the decision or “disguised paternalism”, informed by the doctor’s own subconscious values.

➢ But note that paternalism can be considered wrong even when the outcome for the patient could be conceived of as **good** (at least from a somatic perspective).

Today, we will zoom in and look at debates concerning paternalism and personalised medicine (especially personalised risk scores).

What is personalised medicine?

Personalised medicine can be understood as a practice where:

- “Personalized medicine is an emerging practice of medicine that uses an individual's genetic profile to guide decisions made in regard to the prevention, diagnosis, and treatment of disease. Knowledge of a patient's genetic profile can help doctors select the proper medication or therapy and administer it using the proper dose or regimen. ” NIH National human genome research institute

Vocabulary:

- “*Precision medicine* is a way health care providers can offer and plan specific care for their patients, based on the particular genes, proteins, and other substances in a person’s body. This approach is also sometimes called *personalized medicine* or *personalized care*.” American Cancer Society

What is the purpose of personalised medicine?

”Personalized medicine, often referred to as precision medicine, is an evolving field in which physicians use molecular diagnostic tests to determine which medical treatments and strategies will work best for their patients.

By combining the data from those tests with an individual’s medical history, circumstances, and values, health care providers and patients can develop targeted treatment and prevention plans.”

http://www.personalizedmedicinecoalition.org/Userfiles/PMC-Corporate/file/PMC_Moving_Beyond_Population_Averages_A_Patient-Centered_Research_Agenda_Advancing_Personalized_Medicine1.pdf

Potential benefits of personalised medicine

1. better medication effectiveness, since treatments are tailored to patient characteristics, e.g., genetic profile;
2. reduction of adverse event risks through avoidance of therapies showing no clear positive effect on the disease, while at the same time exhibiting (partially unavoidable) negative side effects;
3. lower healthcare costs as a consequence of optimized and effective use of therapies;
4. early disease diagnosis and prevention by using molecular and non-molecular biomarkers;
5. improved disease management with the help of wearable sensors and mobile health applications;
6. smarter design of clinical trials due to selection of likely responders at baseline

(see Frölich et al 2018)

What are the relevant medical technologies?

"Within the next decade we will see an evolution in medicine as we understand more about the underlying drivers of a patient's condition and their individual response. …This evolution to personalisation in medicine is not new, but is being driven by scientific advances and the development of new technologies, particularly the huge leaps in computing power that have driven the development of **artificial intelligence and data analytics.** Innovations such as 3D printing and circulating tumour DNA analysis are also providing ways of personalising care as never before."



Prof. Dame Sue Hill (Chief Scientific Officer for England)

(https://www.phgfoundation.org/documents/phgf-personalised-medicine-technology-landscape-report-50918.pdf

Personalised risk scores

A personalised risk score (or personal risk information) estimates the probability of future health outcomes for individual patients.

Personalised risk scores are regarded promising, because

- they could help clinicians and other medical practitioners to better predict whether individuals will develop disease
- more effective prevention and treatment options

For more information and concrete examples, see e.g.

Han PKJ, Hootsmans N, Neilson M*, et al* The value of personalised risk information: a qualitative study of the perceptions of patients with prostate cancer *BMJ Open* 2013;**3:**e003226. doi: 10.1136/bmjopen-2013-003226

Edwards AGK, Naik G, Ahmed H, Elwyn GJ, Pickles T, Hood K, Playle R. Personalised risk communication for informed decision making about taking screening tests. Cochrane Database of Systematic Reviews 2013, Issue 2. Art. No.: CD001865. DOI: 10.1002/14651858.CD001865.pub3. Accessed 03 December 2020.

Sceptical approach

According to Stephen John (and others) there are two reasons to be sceptical of claims that imply that personalised risk score technologies will pave the way for effective prevention:

1 - Patients will misunderstand personalised predictions leading them to make worse medical decisions than they otherwise would

2 - Increased knowledge of personalized risk scores will undermine citizens' willingness to contribute to socialized healthcare

John, S. (forthcoming). Artificial Ignorance, Epistemic Paternalism and Epistemic Obligation(Ch.1). In *Epistemic Paternalism Reconsidered,* ed. Axtell and Bernall

“Both concerns, if valid, imply reasons to control the flow of information from personalised medical tests: the first implies that results should be available only when they are interpreted and contextualised by intermediary experts, such as trained genetic counsellors; the second implies reasons against allowing test results to be available at all.” (John forthcoming, p.1)

?

Should we control the flow of information?
Or: how **strongly should** we control the flow of information?

Recall that paternalism is

> “[T]he interference of a state or an individual with another person, against their will, and defended or motivated by a claim that the person interfered with will be better off or protected from harm.” (Dworkin 2014).

So we are back to our original question

When, if ever, are paternalistic interventions justified?

Or: When are the controls on the flow of information on personal health risks permissible?

Break

15 min

Are controls on the flow of information on personal health risks permissible?

We will consider three responses:

(1) Not permissible (Extreme autonomy!)
(2) Permissible (Extreme beneficence!)
(3) Permissible. New framework: obligations.

Response 1.

Controls on the flow of information are **not** permissible at all, because such controls place limits on freedom of inquiry, and placing limits of freedom of inquiry is not in the patients' (epistemic) interests.

- Rejects both the worries expressed by John – 'bad' reasons (even if they turned out to be correct that would not mean that they are overriding reasons)

Response 1: not permissible to control flows of information

- Motivated by anti-paternalism

- According to the strongest form of anti-paternalism – 'hard anti-paternalism' – one must never interfere with any self-regarding choice or action performed by an adult.

- But how compelling is response 1?
  - Can't we imagine situations when we would be better off with filtered information? Or at least *some* form of controls of information?

Response 2.

Controls on the flow of information are permissible, because increased knowledge of personalised risk scores will undermine citizens' willingness to contribute to socialised healthcare.

Discussion from John (forthcoming) spelled out

(1) Principles of justice demand that there a form of socialised healthcare available, where healthcare provision is, at least partially, funded through general taxation.

(2) Us who contribute to socialised healthcare systems do so because such a contribution is in our own interest. There is a good chance that we will need to access such a system in the future!

(3) Point/premise (2) rests on the assumption that we all are on a roughly equal risk of needing healthcare in the future. *But some citizens undersanding their personal risk scores really, really well undermines this assumption.* What seemed like a prudent investement becomes a burdensom subsidy for them!



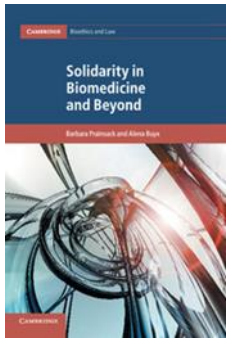
Undermines citizens willingness to contribute to socialised healthcare, which is a serious worry.

“Allowing access to personalised medical results will undermine the solidaristic attitudes necessary for sustaining [socialised healthcare] institutions which are demanded by justice.”

(John (forthcoming), p. 9)

If this is true then some paternalistic interventions on the flow of information are justified

A counter argument against response 2



We might question that personalised medicine *must* undermine solidarity. (Praisanck and Buyx 2017)

"[…] a solidarity-based perspective enables us to see that personalised medicine can be harnessed and understood in a different way to the more individualistic interpretation that emerges when it is mainly considered in the context of genetic and other biomarkers (Hamburg and Collins 2010). And solidarity also gives us the tools to start harnessing this potential." (2017, p.138)

". […] A universal healthcare system organised in a solidaristic spirit would be the best safeguard against the scenario that personalisation would be used to justify undue exclusion, or distributed according to people's ability to pay, rather than their need. " (p. 138)

- So for Prainsack and Buyx, access to personalised risk scores will not automatically jeopardise socialised healthcare (or make people less willing to contribute), insofar as we are striving for a more solidaristic system overall.
- But perhaps maintaining "solidaristic attitudes" does call for *some restrictions* on the flow of information?

→ then Prainsack and Buyx's argument is not all that convincing if mobilised to support *unrestricted* access to personalised risk scores.

Are controls on the flow of information on personal health risks permissible?

We will consider three responses:

~~(1) Not permissible (Extreme autonomy!)~~

~~(2) Permissible (Extreme beneficence/solidarity!)~~

(3) Permissible. New framework: obligations.

Obligations

John (forthcoming).

“(…) debates over epistemic paternalism lead us to problems about how to commensurate freedom and the good. I will now suggest that an advantage of the alternative, “obligation-first” approach is that it avoids this problem.”

Obligations towards ourselves

Self-directed epistemic obligation

- “An obligation to acquire or maintain some epistemic state, derived from a more fundamental ethical obligation.”
- Example: an obligation to become better informed.

But! We can also have an obligation to ***not*** be informed.

- “For example, in virtue of my more general obligations to respect my friend’s privacy, I have an obligation not to read his secret diary which he has left open on the table. I can have obligations to remain ignorant (Manson, 2012).”

[All the direct quotes are from (John, forthcoming).]

Obligations towards others

Epistemic enablement obligations

- “obligations to enable others to meet their epistemic obligations.”

Just as we can have obligations of ignorance, others may have an obligation to enable us to maintain our ignorance.

- “For example, a third party, who knows that I am insatiably (but improperly) curious about my friend’s private life, finds my friend’s diary open on the table. She has, I suggest, an obligation to hide away the diary as a way of enabling me to ensure that I meet my obligation of ignorance.”

[All the direct quotes are from (John, forthcoming).]

Obligations

Citizens have obligations – grounded in their broader duties to support just institutions – not to gain more personalised information.

- “our obligation to support **just institutions** can place limits on our pursuit of our prudential interests “

The state has a correlative duty to enable citizens to fulfill of these obligations.

- “the second obligation follows as a result of States’ duties to cultivate citizens’ dispositions to advance **justice** (Macedo, 1992).”

This is a relational account more related to ethics of care than say rights and duties. There is a focus on justice, equity and fairness and the commitment to these values necessitates obligations that will restrict some actions that the alternative individualistic/ rights philosophy emphasises

Stephen John's conclusion

"[r]ather than conceptualise information control policies in terms of a clash between rights and consequences, we can, instead, think about them as embedded in wider patterns of socioepistemic obligation. Of course, weighing and assessing different obligations is not an easy task, but it is easier than balancing freedom against the good."

Given this justice orientated framework there should be controls on the flow of information

But, note that while he says that epistemic paternalism is not the most useful tool, rather we should think in terms of "interlocking sets of epistemic obligations" that is not to say that all controls on information are just –

"For example, there may be ethical reasons why the State should control the flow of personalised information, maintaining a just healthcare system, but, also, good ethical reasons to worry that these powers might be abused to maintain unjustifiable forms of ignorance. As such, there may well be pressing ethical, political and practical reasons to resist controls on the flow of information."

Discussion 15 min – choose 2 examples

- Is it an act if medical paternalism? Does it meet the two criteria? A) designed to limit autonomy and B) reliably beneficence based
- Is it justifiable? Identify and specify the value tension. Give ethical reasons for and against

- Ex 1 – A blood transfusion to save the life of an adult Jehovah's witness without their informed consent
- Ex 2 – Compulsory bicycle helmets for adults
- Ex 3 – Increase VAT by 300% on un-healthy produce e.g. candy, soda, crisps..
- Ex 4 – The doctor deems the patient too frail to handle the cancer diagnosis and decides not to tell the patient. The patient is in her 90s and expected to die within 6 months due to an unrelated disease. The patient is cognitively clear and has full decision making capacity.
- Ex 5 – The doctor is convinced that the new surgical technique she has developed is superior to the available alternative techniques and methods. It has been tested rigorously and there is evidence to support it's reliability. It's a good technique but not as outstanding as the doctor believes. She tells the patient about all the alternatives but emphasises the benefits of her own. She is not neutral and it is possible that she manipulates the patient a little bit but she is convinced that her technique would be the best choice for this patient. The patient goes with the doctors recommendation and has the surgery. The patient recovers quickly and is very happy.

Summary

- We have been looking at paternalism, i.e. when is it justified for a state to interfere with individual's life (freedom/autonomy, and wellbeing)
- We studied paternalism in placing controls on the flow of information regarding personalised risk scores
- And we looked at an obligations–orientated framework (which places more importance on justice over autonomy/freedom)



The questions at the essay writing session will **not** involve any detailed questions on Stephen John's arguments.

Further readings and references from the lecture

Begon, Jessica. "Paternalism." Analysis 76, no. 3 (2016): 355-373.

Dworkin, Gerald, "Paternalism", *The Stanford Encyclopedia of Philosophy* (Fall 2020 Edition), Edward N. Zalta (ed.), URL = <https://plato.stanford.edu/archives/fall2020/entries/paternalism/>.

Fröhlich, Holger, Rudi Balling, Niko Beerenwinkel, Oliver Kohlbacher, Santosh Kumar, Thomas Lengauer, Marloes H. Maathuis et al. "From hype to reality: data science enabling personalized medicine." BMC medicine 16, no. 1 (2018): 150.

Haybron, D.M. and A. Alexandrova. 2013. Paternalism in economics. In Paternalism: Theory and Practice, ed. C. Coons and M. Weber, 157–77 Cambridge: Cambridge University Press.

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Pirmohamed, M. (2001). Pharmacogenetics and pharmacogenomics. *British Journal of Clinical Pharmacology*, *52*(4), 345–347. https://doi.org/10.1046/j.0306-5251.2001.01498.x

Salari, K. (2009). The Dawning Era of Personalized Medicine Exposes a Gap in Medical Education. *PLOS Medicine*, *6*(8), e1000138. Retrieved from https://doi.org/10.1371/journal.pmed.1000138